



Fiona Stanley Fremantle Hospitals Group

OFFICIAL

Guideline

Naloxone prefilled take home syringes (Prenoxad®) and nasal delivery device (Nyxoid®)

Reference #: FSFHG-HW-GUI-0029

Scope

Site	Service/Department/Unit	Disciplines
Fiona Stanley Hospital Fremantle Hospital	Hospital Wide	Medical, Nursing and Pharmacy

1. Introduction

This guideline outlines recommendations for the supply of take-home naloxone to patients who present using prescribed or non-prescribed opioids and are deemed to be at risk of an opioid overdose. Supplying naloxone with appropriate psychoeducation to patients on leave or discharge from an inpatient unit enables the opportunity to reverse an opioid overdose with naloxone should this be required in the future. This approach is consistent with harm reduction principles.

2. Terminology

Naloxone	Competitive opioid antagonist which may be administered to reverse the effects of an opioid overdose.
Opioid	A substance that interacts with opioid receptors. Often utilised for analgesic, gastrointestinal and respiratory effects. Opioids may be prescribed for therapeutic purposes or are used recreationally. There are two types of opioid medicines: occurring naturally and made from the opium poppy (e.g. codeine, morphine and heroin)

	created synthetically (e.g., pethidine and fentanyl)
Opiate Substitution Therapy (OST)	Includes methadone syrup/solution and buprenorphine containing sublingual tablets, films, and injections such as Subutex®, Suboxone®, Buvidal® and Sublocade® for the treatment of opioid dependency.

3. Guideline

3.1. Take-Home Naloxone program

Under the national Take Home Naloxone program, naloxone is available:

- free of charge and without a prescription across all participating pharmacies in Australia.
- over the counter from non-participating Take-Home Naloxone program pharmacies for a fee.
- under the Take Home Naloxone program from a variety of locations including:
 - community and hospital-based pharmacies
 - alcohol and other drug treatment centres
 - needle and syringe programs
- A list of Take Home Naloxone registered service providers by state/territory is available on the Pharmacy Programs Administrator website and can be accessed [here](#).

3.2. Risks for opioid overdose

- The patient:
 - has a history of previous opioid overdose using prescribed or non-prescribed substances.
 - injects opioids e.g. heroin, “home bake”, fentanyl, prescription opioids
 - uses multiple substances with sedative potential (e.g. alcohol, benzodiazepines, gabapentinoids)
 - has a low tolerance (opioid use after a period of abstinence such as following hospitalisation, or period of incarceration)
 - uses opioids alone. Whilst the person is not able to administer their own naloxone in an overdose, it is important that naloxone is available as it may allow a bystander to assist.
 - has concurrent chronic medical conditions.
 - is opioid naïve and has been prescribed opioids that may represent a risk.

- Concerns have been raised by treating clinical staff and/or family/significant others of the risk of future opioid overdose.

3.3. Patient inclusion criteria

- aged 16 years and over.
- has completed psychoeducation on the use of naloxone.
 - Family, friends, significant others who may be in attendance with the patient can also receive the psychoeducation session.
- at risk of future opioid overdose.
- currently receiving OST
- has verbally provided consent to be supplied with Take Home Naloxone products

3.4. Exclusion criteria

- Known allergy to naloxone
- Under the age of 16 years AND is unable to demonstrate sufficient knowledge on the use of naloxone to be safely given a supply.
 - Naloxone may still be supplied if education can be provided to the next of kin or another person who is accompanying the patient and is likely to witness an overdose
- has not verbally consented to having a supply.

3.5. Dose and Administration

3.5.1. Prenoxad®: naloxone 1mg/mL prefilled syringe (each 2mL syringe contains 5 doses of 400microgram per dose)

- Give 0.4mL (to first black line on syringe) via intramuscular injection into the outer thigh or upper arm muscle
- Repeat every 2-3 minutes until ambulance arrives or patient regains consciousness.
- Prenoxad® is for single patient use (multiple doses from the device may be administered to the same patient) and any used devices should be discarded.

3.5.2. Nyxoid®: naloxone 1.8mg/actuation nasal spray (2 devices each administering 1 dose)

- Administer 1 spray (1.8mg) into one nostril.
- If needed, a second dose (using a new device) may be sprayed into the other nostril after 2-3 minutes. Additional doses may be given if available and needed, until adequate emergency medical care is available.

- When administering the dose of Nyxoid®, ensure the nose is clear, insert the nozzle into nostril and press firmly on the plunger until it clicks to give the dose.

3.6. General Counselling Points

- Staff providing Take Home Naloxone should also provide general education on:
 - recognising and responding to signs and symptoms of opioid overdose
 - obtaining emergency medical care immediately. Naloxone treatment is not a substitute for emergency medical care
 - the duration of action of naloxone. It takes 2 to 5 minutes to start working and effects last for 30 to 90 minutes. The duration of action of most opioids may exceed that of naloxone resulting in a return of respiratory and/or central nervous depression after an initial improvement of symptoms. Refer to [Appendix 2](#) for the duration of action of common opioids.

3.7. Supply and dispensing

Patients who meet the inclusion criteria may be supplied the naloxone 1mg/mL prefilled syringe (Prenoxad®) or the naloxone 1.8mg/0.1mL nasal spray (Nyxoid®) according to the flowchart in [Appendix 1](#).

No more than 2 product packs may be supplied at one time (either two packs of the same product or one pack of two different products).

- Before patients can be supplied Take Home Naloxone they must have received education on its use and other general counselling as detailed above. It is the responsibility of the person providing the Take Home Naloxone to provide education to the patient.
- Take Home Naloxone must be supplied by staff members who have the knowledge and skills to deliver education to consumers for the use of Prenoxad® prefilled syringes or Nyxoid® nasal delivery system. Staff must have completed training provided by the WA Mental Health Commission (MHC) on the provision of Take Home Naloxone products. In the first instance this may include:
 - Emergency Department (ED) nursing staff
 - ED doctors
 - Drug and Alcohol doctors or nurses
 - Other pharmacy and medical staff as appropriate.

- Any registered or enrolled nurses supplying Prenoxad® or Nyxoid® must comply with the corresponding [Department of Health \(DOH\) Structured Administration and Supply Arrangement \(SASA\)](#)

3.7.1. At FSH:

- Take Home Naloxone is available in specific Med stations (ED Assessment), Ward 5A, Ward 6D, State Rehabilitation Service (SRS) 1A and Mental Health A).
- A nurse or pharmacist must access the med station to enable supply to the patient.

3.7.2. At FH and FSH-CH

- FH and FSH-CH are registered Authorised Alternate Suppliers of Take-Home Naloxone. Any supply must be recorded and reported to the MHC at the start of each month.
- At FH the monthly reporting of any supply of Take Home Naloxone to the MHC is to be completed by the Dual Diagnosis Nurse(s) at FH.
- At FSH-CH the monthly reporting of any supply of Take Home Naloxone to the MHC is to be complete by the AOD CNS at FSH-CH
- At FH Take Home Naloxone products are available in the MH Triage S4R cupboard
- At FSH-CH Take Home Naloxone products are available in the S4R cupboard on the Alcohol and Other Drug (AOD) Behavioural and Withdrawal Management Ward.
- Any supplies of Prenoxad® or Nyxoid® must be recorded in the corresponding register. Date of supply, patient details, staff designation and quantity of product supplied must be recorded.
- If the patient consents, the following two questions should also be recorded:
 - Was the supply a refill or initial supply?
 - If a refill, what was the reason for refill?
 - Used Take Home Naloxone on self
 - Used Take Home Naloxone on other
 - Lost, damaged or expired Take Home Naloxone
 - Given to someone else to keep

If staff members with the knowledge and skills required are not available, or the patient has presented at Fremantle Hospital Mental Health Crisis Centre solely for the purpose of accessing naloxone, they should be advised to go to a community alcohol and drug service or participating Take Home Naloxone community

- Written patient information must be provided and can be accessed online:
 - [Take Home Naloxone – what you need to know](#)
 - [Prenoxad® Administration](#)
 - [Nyxoid® Administration](#)

3.8. Adverse effects

- The abrupt withdrawal of opioids in physically dependent persons typically results in an opioid withdrawal syndrome. Symptoms include:
 - irritability, confusion
 - nausea, vomiting
 - gastrointestinal pain
 - muscle spasms
 - tachycardia
 - increased blood pressure
 - pyrexia
 - yawning
- Possible adverse effects caused by naloxone are illustrated in Table 1.

Table 1: Potential adverse effects of naloxone and their incidence

Incidence	Potential Adverse Effects
• Very Common ≥10% incidence	• nausea
• Common ≥ 1% to <10% incidence	• dizziness • headache • tachycardia • hypotension • hypertension • injection site pain

Incidence	Potential Adverse Effects
Uncommon ≥ 0.1% to <1% incidence	- tremor, sweating. - arrhythmia - bradycardia - hyperventilation - diarrhoea
Rare ≥ 0.01% to < 0.1% incidence	seizures
Very rare <0.01% incidence	- cardiac fibrillation - cardiac arrest - pulmonary oedema (may occur more frequently in patients with pre-existing cardiovascular disease or those receiving concomitant medication with adverse cardiovascular effects)

3.9. Training and resources

- The MHC provides training on recognising and responding to opioid overdose. Course information is available on the [Alcohol and other drug training calendar](#) or by emailing the MHC at naloxone@mhc.wa.gov.au

4. Compliance/performance monitoring

- Compliance with this guideline will be monitored by the Alcohol and Other Drugs Service (AODS) Clinical Nurse Coordinator and the SMHS MH Nurse Director via routine clinical incident monitoring processes.
- The FH & FSH Senior Pharmacists will monitor dispensing practices and escalate any concerns to the AODS.

5. Related policy documents

- WA Health:
 - [High Risk Medication Policy](#) MP0131/20
- FSFHG:
 - Medication Administration
 - Alcohol and Other Drugs Management of addiction, toxicity and withdrawal
 - High Risk Medications

6. Other related documents

- [SASA 044/1-2025 Supply of naloxone by Registered Nurses for emergency reversal of drug overdose](#)
- [SASA 045/1-2025 Supply of naloxone by Enrolled Nurses for emergency reversal of drug overdose](#)

7. Related standards

- NSQHS Standards
 - Clinical Governance
 - Partnering with Consumers
 - Medication Safety
 - Comprehensive Care
 - Recognising and Responding to Acute Deterioration

8. References

1. Mental Health Commission 2019. [Western Australian Mental Health, Alcohol and Other Drug Services Plan 2015-2025 \(Plan\)](#) Mental Health Commission, Government of Western Australia.
2. Department of Health, 2024, Take Home Naloxone Program, [Take Home Naloxone program | Australian Government Department of Health and Aged Care](#)
3. Australian Medicines Handbook. (2024). Opioid Analgesics. In *Australian Medicines Handbook*. Retrieved November 19, 2024, from: <https://amhonline-amh-net-au.smhslibresources.health.wa.gov.au/chapters/analgesics/drugs-pain-relief/opioid-analgesics?menu=vertical>
4. Choo, C. (2009) 'Medications Used in Opioid Maintenance Treatment', *U.S Pharmacist*, 34(11), pp. 40–53.

9. Authorisation

EXECUTIVE SPONSOR: Nurse Director, Service 5					
Version	Date Issued	Compiled/Revised by	Committee/Consumer Group consulted	Endorsed by	Revision due
1	05/2020	Clinical Nurse Consultant, Alcohol and Other Drugs Service	NMPGC	FSFHG Policy Committee	04/2024
2	07/2024	Clinical Nurse Consultant, Alcohol and Other Drugs Service	NMPGC, DTC and S5 SQR Committee	FSFHG Policy Committee	07/2028
2.1	08/2024	CNS Clinical Improvement	Minor correction with no change to review date		07/2028
3	07/2025	FSH-CH MH Team Lead Pharmacist	DTC, NMPGC, S5 SQR Committee	FSFHG Policy Committee	07/2029
4	10/2025	FSH-CH MH Team Lead Pharmacist	DTC Minor Amendment	FSFHG Policy Committee	07/2029

10. Appendices

10.1. Appendix 1: Flowchart for dispensing of naloxone at FSH (excluding FSH-CH).

24 hours a day

Patient identified as at risk of opioid overdose and meeting inclusion criteria or on OST.

Offer the provision of naloxone.

Prenoxad® and Nyxoid® is provided

to a patient only after a staff member with appropriate knowledge and skills provides the patient with an education session and a written information sheet.

Complete:

1. Medical record entry
2. If patient consents to de-identified data collection, contact AOD CNC for recording of details

10.2. Appendix 2 Opioid duration of action^{3,4}

Opioid	
Buprenorphine	6 to 8 hours*
Codeine	3 to 4 hours
Fentanyl	1 to 2 hours*
Heroin	4 to 6 hours
Hydromorphone	2 to 4 hours
Methadone	8 to 24 hours
Morphine	IR: 2 to 3 hours MR: 12 to 24 hours
Oxycodone	IR: 3 to 4 hours MR: 12 hours
Pethidine	2 to 3 hours
Tapentadol	IR: 4 to 6 hours MR: 12 hours
Tramadol	IR: 3 to 6 hours MR: 12 hours
IR: immediate release, MR: modified release	
*post single dose, not applicable to transdermal preparations	